



POLICY

Further Opinions

Scope (Staff):	All CAMHS staff
Scope (Area):	All CAMHS directorates

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This document should be read in conjunction with this [disclaimer](#) and the [Patient Rights under the Mental Health Act Policy](#)

Aim

All consumers, their families, carers and other nominated persons will be informed of their right to request a further opinion as outlined in the [Mental Health Act 2014 \(WA\)](#) (MHA). This includes where an unresolved difference of opinion among clinicians, families or other relevant agencies exists.

This policy outlines the requirements, processes and support mechanisms related to the provision of further opinions.

Risk

Failure to adhere to this policy may result in health impacts to consumers as a result of misdiagnosis.

Failure to provide further opinions in a timely and supportive manner may result in a risk to organisational reputation.

Failure to provide further opinions for involuntary and mentally impaired accused patients may constitute a breach of the MHA.

Background

CAHS is committed to ensuring that consumers are informed of their rights as per the [Australian Charter of Healthcare Rights](#) and the MHA. Consumers and their families can expect to be supported and involved in matters relating to diagnosis, treatment

and care planning to ensure a shared understanding. Clinicians have a duty to inform consumers of their rights and support them in exercising these rights.

CAMHS applies a multidisciplinary approach in all assessment, treatment planning and intervention to ensure rigor and collaboration. Consumers are informed of this throughout the service journey.

Definitions

Further opinion: A medical opinion provided by a second medical specialist/expert or after an initial opinion provided by a medical specialist/expert. Often referred to as a 'second opinion' in the context of involuntary patients.

The extent of further opinions will vary. A further opinion may comprise the following:

- A medical record and medication chart review by a non-treating psychiatrist within the team; or
- A multidisciplinary team meeting with a psychiatrist in another team/service; or
- A complete re-assessment and re-testing.

Independence: The degree of independence is to be informed by the options available and is to optimise patient choice as much as feasible. The benchmark for independence is external to the treating service¹, i.e. external to CAMHS. In some cases, a further opinion from a CAMHS consultant psychiatrist external to the current CAMHS treating team may be deemed satisfactory by the legal guardian.

Involuntary patient: A person subject to an involuntary treatment order under the MHA, including inpatient treatment orders and community treatment orders.

Legal guardian: A person other than a parent of a minor who has been granted legal authority to assume care of, and make decisions for, another individual.

Medical specialist: In this policy, a medical specialist refers to a consultant psychiatrist.

Other relevant agencies: A stakeholder, other than the consumer or their parent or legal guardian, who may dispute the diagnosis, treatment and/or management of a child or young person. Examples include the Department of Communities Child Protection and Family Support acting as a consumer's legal guardian.

Voluntary patient: An active client of CAMHS who is receiving, or proposed to receive, treatment from CAMHS, but who is not:

- An involuntary patient; or
- A mentally impaired accused required under the [Criminal Law \(Mental Impairment\) Act 2023](#) (WA) to be detained at an authorised hospital.

¹ Dr Nathan Gibson (Chief Psychiatrist, Office of the Chief Psychiatrist). Electronic letter to Dr Jacques Esterhuizen (A/Head of Department PCH, Child and Adolescent Mental Health Service). 14 April 2021. 1 leaf.

Key points

- Provision of further opinions for voluntary and involuntary mental health patients will be in accordance with the MHA.
- Information regarding accessing further opinions is available on the CAMHS Internet and welcome packs provided to children, young people and their families on admission to the service.
- Further opinions may be initiated at any time while the consumer is active within CAMHS.
- Further opinions may be requested and accessed for a variety of reasons. These may include:
 - Requests for diagnostic clarification; or
 - Requests for advice regarding clinical management of acute or chronic high-risk issues (e.g. verification of the appropriateness of the proposed treatment considering 'treatment resistance' with various complex needs); or
 - Applicable statutory requirements under the MHA.
- Further opinions may be requested by:
 - The consumer; or
 - The consumer's parent/legal guardian; or
 - An Advocate from the Mental Health Advocacy Service (MHAS) on behalf of the consumer or parent/legal guardian; or
 - A treating psychiatrist who, during the course of treatment, deems it appropriate to seek a further opinion; or
 - The Chief Psychiatrist.
- A need for a further opinion may be discussed verbally however must be requested in writing to the treating psychiatrist.
- Involuntary and Mentally Impaired Accused patients are entitled to a further opinion unless MHA s.183 request for additional opinion applies.
- The further opinion process will be guided by principles of transparency and child and family centred practice at all times.
- The treating service will ensure that barriers to health equity are reduced, particularly regarding facilitating access to further opinions for Aboriginal people, people from culturally and linguistically diverse (CaLD) backgrounds, and people living in lower socioeconomic conditions.

Process

The following process supports a timely, collaborative and consistent approach to requests for further opinions. This process is illustrated in Appendix A.

When a further opinion is requested, the treating psychiatrist or delegate must, as soon as practicable:

1. Notify the HOD of the request for further opinion.
2. Arrange a **family meeting**, discuss concerns, determine the basis for seeking a further opinion and, where indicated, present a range of options for how that further opinion may be sought.

Depending on the consumer and legal guardian's perspective on the level of independence, options offered for a further opinion may range from:

- A psychiatrist **within the same CAMHS team/site**; or
- A psychiatrist from **a different team/site but within the same directorate** of CAMHS (the psychiatrist must not be a member of the treating team); or
- A psychiatrist in a **different CAMHS directorate** (e.g. Community or Acute CAMHS if the consumer is an inpatient); or
- Where practicable, a psychiatrist from a **different health service provider within WA** (e.g. Fiona Stanley Hospital, WA Country Health Service, WA Eating Disorders Outreach and Consultation Service). These requests should be handled on a case-by-case basis, given known capacity limitations (i.e. availability of appropriately qualified psychiatrists); or
- A **private psychiatrist**.

The cost of a further opinion from a private psychiatrist is to be borne by the legal guardian if they express a preference for a private psychiatrist to provide the second opinion.

Children and young people are a specialised group, and where practicable, identifying an appropriately qualified child and adolescent psychiatrist is advisable.

The advantages and disadvantages of the options above (e.g. degree of independence, type of further opinion e.g. chart review vs full re-testing, timeliness, financial cost) should be discussed with the consumer and/or legal guardian so that they may make an informed decision. The discussion and outcomes are to be documented in the medical record, including options presented, decisions made and rationales. Refer to the resources section of this policy for ideas on how to support this discussion.

3. Information and access to advocacy services available within CAMHS and more broadly must be offered. This includes cultural, peer or family peer support where available and appropriate, engagement with the Mental Health Advisory Service, Health Consumer Council or other services.

4. Notify the Head of Department (HOD) of the option chosen for the further opinion by the consumer.
5. Facilitate the further opinion as indicated.
 - Where the consumer/legal guardian's preferred option is a further opinion from a psychiatrist from another team, site or health service, the HOD or delegate (e.g. Head of Service (HOS) will contact the HOD or HOS of the other site/service and organise for an available psychiatrist to provide the further opinion.
 - Where the consumer/legal guardian's preferred option is a further opinion from a private psychiatrist, it is the legal guardian's responsibility to engage any suitably qualified psychiatrist.

Within reason, the treating psychiatrist or delegate may provide support to identify a suitable private psychiatrist, if acceptable to the legal guardian.

6. The written copy of the further opinion report outlining the decisions and rationale is to be provided to the consumer and their family.
7. Arrange a follow-up family meeting on receipt of a further opinion. The treating psychiatrist or delegate will assess any additional information and ascertain with the consumer/legal guardian a final outcome based on the opinion offered.
8. Should there be ongoing dispute between the consumer/legal guardian and the treating psychiatrist, despite the further opinion, or if the further opinion is not accepted by either party, then the treating psychiatrist is to escalate the matter to the HOD.

Refusal of request for further opinion

In some instances, the treating psychiatrist and/or HOD and/or Medical Co-Director may determine that a further opinion is not appropriate. For example, where a further opinion has already been provided and a request for an additional further opinion is made.

When an additional request is not warranted,² an automatic refusal of a request for a further opinion is not recommended. A family meeting with the treating psychiatrist and the HOD is strongly encouraged.

In the event that a request for a further opinion is declined, the treating psychiatrist, as soon as practicable, must record the decision and the reasons and provide a copy to the legal guardian and document the outcomes in the clinical record.

Where the request pertains to an involuntary patient and the decision is to decline the request, the treating psychiatrist must complete the [Chief Psychiatrist Notification From – Refusal of further opinion request by psychiatrist](#) and provide a copy to the chief Psychiatrist.

² Refer to Chief Psychiatrist's Guidelines for Clinical Practice (2015) for scenarios where this may apply.

Documentation

All further opinion requests are to be documented in the medical record by the treating psychiatrist or delegate.

Documentation must include:

- Receipt of written request for a further opinion;
- Family meeting discussions and outcomes, including decision on options; and
- Reasons for any decision.

The Further Opinions Register will be maintained by the HOD in consultation with the treating Psychiatrist.

Roles and Responsibilities

The **Heads of Department** are responsible for:

- Maintaining a register of further opinions.
- Supporting the treating psychiatrist in finding suitable psychiatrists to provide further opinions.

The **Treating Psychiatrist** is responsible for:

- Responding to requests for further opinions.
- Notifying the HOD of requests for further opinions and actions taken.
- Providing a follow up meeting with the consumer and family on receipt of further opinions report.
- Reviewing the Treatment Support and Discharge Plan (TSDP) in collaboration with treatment team, consumer and family in consideration of the recommendations outlines in the further opinion report.

Any staff member receiving a request for a further opinion is responsible for escalating the request to the treating psychiatrist in a timely manner.

Consumer Engagement Officers, Aboriginal Mental Health Workers and Peer Support Workers are responsible for:

- Providing information regarding further opinions.
- Attending education and training regarding further opinions.
- Supporting consumers and their families in accessing further opinions.

CAMHS Safety and Quality Unit is responsible for reviewing the further opinions registers and compiling a report to be tabled at CAMHS Executive Committee on annual basis.

CAMHS Executive Committee is responsible for monitoring the governance of further opinions and escalating as appropriate.

Monitoring and reporting

A register of all requests for further opinions and outcomes will be maintained by CAMHS HODs. On an annual basis CAMHS Safety and Quality Unit will undertake a review and analysis of the data and provide a written report to be tabled annually at the CAMHS Executive Committee.

CAMHS will collaborate with the Chief Psychiatrist in undertaking monitoring activities related to the treatment and care of mental health patients within Western Australia as prescribed by the MHA s. 515. Monitoring activities include:

- Without Notice Reviews (MHA s. 521)
- Clinical Standards and Service Reviews
- Targeted Reviews
- Thematic Reviews of Mental Health Services

References and related external legislation, policies, and guidelines
<u>Mental Health Act 2014 (WA)</u>
<u>Criminal Law (Mental Impairment) Act 2023</u>
<u>Carers Recognition Act 2004 (WA)</u>
<u>MP 0155/21 - Statewide Standardised Clinical Documentation for Mental Health Services Policy</u>
<u>Chief Psychiatrist's Guidelines</u>
<u>Refusal of Further Opinion Request by Psychiatrist</u>
<u>Chief Psychiatrist's Standards for Authorisation of Hospitals Under the Mental Health Act 2014 (WA)</u>
<u>Chief Psychiatrist's Review into the Treatment of Ms Kate Savage by Child and Adolescent Mental Health Services</u>
Related CAHS internal policies, procedures and guidelines <i>(if required)</i>
<u>CAMHS Patient Rights Under the Mental Health Act</u>
<u>CAMHS Ward 5A Model of Care</u>
<u>CAHS Paediatric Goals of Care Policy</u>

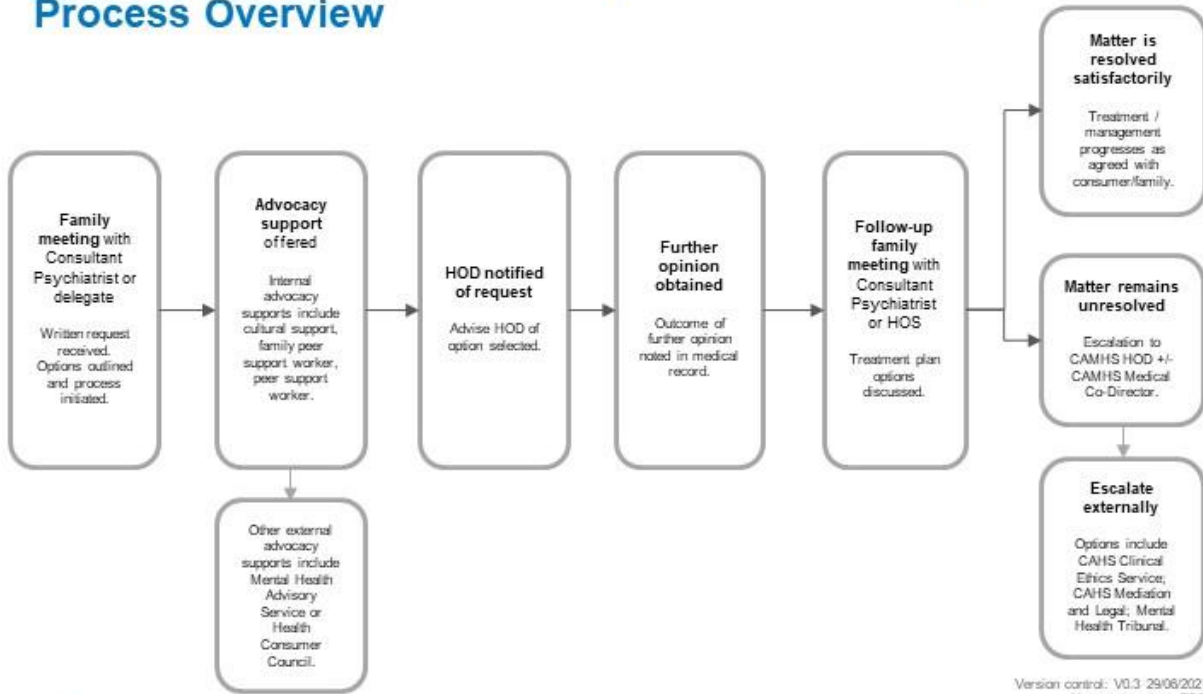
Useful resources (including related forms) <i>(if required)</i>
Advocacy Services and Peak Bodies
Australian Charter of Healthcare Rights
Health Consumers Council WA
Mental Health Advocacy Service
Mental Health Commission Information for Voluntary Patients in Hospital
Mental Health Commission Mental Health Act 2014 Further Opinions Fact Sheet
Office of the Chief Psychiatrist of Western Australia Clinicians' Practice Guide to the Mental Health Act 2014
The Royal Australian and New Zealand College of Psychiatrists Find a psychiatrist

This document can be made available in alternative formats on request.

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Appendix 1

CAMHS Provision for Further Opinions for Voluntary Patients: Process Overview



Compassion

Excellence

Collaboration

Accountability

Equity

Respect

Healthy kids, healthy communities